

# Suprapubic Catheter Placement

Patient Information · What to expect before your procedure

WARWIKI

A suprapubic catheter, also called a suprapubic tube or “SPT,” is a soft tube that drains urine from your bladder. It goes through a small opening in your lower belly, just above the pubic bone, instead of through the urethra. This handout explains why it is placed, how to prepare, and what to expect.

## About This Procedure

The bladder stores urine. Sometimes urine cannot drain the usual way, or the urethra needs to rest or be bypassed. Then a catheter can be placed straight into the bladder through the lower belly. Because it avoids the urethra, it is often more comfortable for long-term use.

**Your care team may suggest a suprapubic tube to:**

- Drain the bladder when you cannot empty it on your own
- Rest or protect the urethra after surgery or an injury
- Give long-term drainage, for example with a nerve-related bladder condition
- Avoid the irritation of a long-term catheter in the urethra
- Allow sex, and allow the tube to be capped to test whether you can urinate

**Other choices:** if you can learn it, emptying the bladder yourself a few times a day with a thin tube (intermittent self-catheterization) is usually preferred, because nothing stays in. A catheter left in the urethra is another choice, but for long-term drainage a suprapubic tube is usually preferred. Your team will help you weigh these.

## Is It Safe?

Yes. This is a common and generally safe procedure. Your bladder is filled so it rises up against the belly wall, away from the bowel. Your team often uses ultrasound, a small camera, or both to guide the tube. A little bleeding, mild discomfort or bladder cramps are common and settle quickly. Injury to the bowel is rare. Belly or pelvic surgery in the past needs extra planning.

## Learn the Terms

### Suprapubic

Above the pubic bone, in the lower belly. This is where the tube comes out.

### Bladder

The organ that stores urine until you empty it.

### Catheter

A soft tube that drains urine.

### Urethra

The tube that normally carries urine out of the body. A suprapubic tube bypasses it.

### Local anesthetic

Numbing medicine placed in the skin so the area does not hurt.

### Sedation

Medicine to relax you. General anesthesia makes you sleep.

### Ultrasound

Sound-wave pictures used to guide the tube in safely.

### Bladder spasm

A sudden cramp of the bladder. It is common afterward and can be treated.

**Will it hurt?** Some placements use numbing medicine, with or without sedation. Others are done under general or spinal anesthesia. Your team will tell you which. Some bladder cramping or aching at the site afterward is normal.

**First days**

Mild aching, cramps and a little blood in the urine.

**Next weeks**

The opening heals around the tube. Keep the skin clean.

**First change**

Done by your team once the opening has healed. They set the date.

**After that**

Regular changes, as your team advises.

**How to Get Ready**

- **Come with a full bladder if asked.** It makes placement safer.
- **Follow any rules about eating and drinking** if you will have sedation or anesthesia.
- **Arrange a ride home** if you will be sedated.

**Tell your team ahead of time if you:**

- Have had **belly or pelvic surgery** or radiation
- Take a **blood thinner** or have a bleeding problem
- Could be **pregnant**, or have an infection or a latex allergy

**What Happens During the Procedure**

- 1 You get numbing medicine or anesthesia, and the lower belly is cleaned.
- 2 Your bladder is filled if it is not already full.
- 3 With guidance, a small opening is made just above the pubic bone, and the tube is passed into the bladder.
- 4 A small balloon holds the tube in place. It is joined to a drainage bag.

**After the Procedure**

- Your team will **show you how to care for the tube and skin** before you go home.
- Urine in a catheter often has germs. Cloudy urine alone is common and usually does not need antibiotics.

**Call the office that placed your tube if you have:**

- A tube that **falls out**. Call right away: the opening can close quickly. Do not force a tube back in.
- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Pain in your side or back, or new confusion
- Redness, swelling or pus around the tube

**Go to an emergency department if:**

- No urine drains and your belly feels full or painful
- Heavy bleeding, or urine like red wine or with clots
- You have a spinal cord injury and get a pounding headache or sweating
- You cannot reach the office and are worried

**Three Things to Remember**

1. A suprapubic tube drains your bladder through the lower belly, bypassing the urethra.
2. Your team does the first tube change once the opening has healed.
3. If the tube falls out, get help the same day. The opening closes quickly.